

SIMULATED INCIDENT REPORT — SYNTHETIC / FICTIONAL DATA
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CONFIDENTIAL — INCIDENT RESPONSE REPORT

Meridian Health Partners

Ransomware Intrusion and Data Exfiltration Incident

Report Reference:	MHP-IR-2026-0143
Prepared by:	Aegis Cyber Response Group (fictional entity)
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Distribution:	IR Steering Committee, Executive Leadership, Legal Counsel, Cyber Insurance Carrier

1. Executive Summary

Between June 21, 2026 and July 6, 2026, Meridian Health Partners (“MHP” or “the organization”), a twelve-site healthcare delivery network headquartered in Columbus, Ohio, experienced a significant cybersecurity incident resulting in unauthorized access to internal systems, theft of sensitive data, and the deployment of ransomware across a substantial portion of the corporate and clinical network. The intrusion was attributed with moderate-to-high confidence to a financially motivated threat actor group tracked internally as SILENT WARD, whose tooling and operational patterns are consistent with affiliate structures observed in the broader ransomware-as-a-service ecosystem.

The intrusion began with a spearphishing email delivered to a Revenue Cycle Management (RCM) department employee, which led to the execution of a malicious macro-enabled document and the establishment of an initial foothold on a single workstation. Over the following eleven days, the threat actor conducted internal reconnaissance, harvested credentials, escalated privileges to Domain Administrator, and moved laterally across 47 hosts spanning corporate IT, clinical documentation systems, and a subset of network-attached imaging infrastructure. Approximately 340 GB of data, including patient billing records, human resources files, and internal financial documents, was staged and exfiltrated to external cloud storage prior to the detonation of ransomware payloads across file servers, domain controllers, and endpoint systems on the morning of July 6, 2026.

The incident resulted in the encryption of approximately 1,200 endpoints and 18 servers, disruption of electronic health record (EHR) access at four clinical sites for a period of 36 to 52 hours, and diversion to manual/paper-based clinical workflows during the recovery window. MHP engaged Aegis Cyber Response Group within four hours of the initial ransom note discovery. Containment was substantially achieved within 30 hours of engagement, and full operational recovery of priority clinical systems was completed within seven calendar days through a combination of clean backup restoration and rebuild of affected domain infrastructure.

This report documents the technical findings of the investigation, the timeline of attacker activity, the scope of affected systems and data, the actions taken to contain and remediate the incident, and recommendations to reduce the likelihood and impact of similar incidents in the future. A separate, restricted-distribution technical appendix contains indicator-level detail suitable for threat intelligence sharing.

2. Incident Classification and Scope

Attribute	Detail
Incident Type	Ransomware deployment with preceding data exfiltration (double extortion)
Threat Actor	SILENT WARD (unattributed affiliate, RaaS-consistent tooling)
Initial Access Vector	Spearphishing attachment (malicious macro document)
Sectors/Systems Affected	Corporate IT, Revenue Cycle Management, HR, subset of clinical documentation and imaging network segments
Estimated Dwell Time	11 days (June 21 – July 6, 2026)
Data Exfiltrated	~340 GB (billing records, HR files, internal financial documents)
Systems Encrypted	~1,200 endpoints; 18 servers (including 2 domain controllers)
Clinical Impact	EHR access disrupted at 4 of 12 sites for 36–52 hours; downtime procedures activated

Attribute	Detail
Regulatory Considerations	Potential HIPAA breach notification obligations under review by Legal/Compliance
IR Firm Engaged	Aegis Cyber Response Group (fictional)
Law Enforcement Notification	Filed with FBI Cyber Division field office (per standard MHP incident policy)

This report covers the period from initial compromise through the conclusion of active containment and eradication activities. Longer-term remediation workstreams (identity architecture overhaul, network segmentation project) are tracked separately under MHP's post-incident improvement plan and are summarized at a high level in Section 10.

3. Timeline of Events

All times are Eastern Daylight Time (EDT) unless otherwise noted. Timestamps reflect a reconstruction based on endpoint telemetry, firewall logs, Active Directory event logs, and cloud storage access logs reviewed during the investigation.

Date/Time	Event
Jun 21, 08:47	Spearphishing email delivered to RCM billing specialist mailbox; sender spoofed a known clearinghouse vendor domain with a single-character transposition.
Jun 21, 09:14	Recipient opens attached invoice document and enables editing/macros at the prompt of an embedded lure image.
Jun 21, 09:15	Malicious macro executes; a PowerShell-based downloader retrieves a second-stage payload from an attacker-controlled server.
Jun 21, 09:16	Loader establishes a scheduled task and a registry Run key to maintain persistence on host RCM-WKS-114.
Jun 21, 09:20 – 22:00	Beaconing activity from RCM-WKS-114 to external infrastructure over HTTPS; low-and-slow cadence.
Jun 22 – Jun 24	Local and domain reconnaissance from RCM-WKS-114 using native Windows utilities and LOLBins.
Jun 25, 02:11	Credential material harvested from LSASS process memory on RCM-WKS-114.
Jun 26	Actor targets over-privileged shared service account (svc-rcmscan); Kerberos ticket material captured and offline-cracked over 48 hours.
Jun 28, 03:40	Privileged service account credentials used to authenticate to jump host (IT-JUMP-02) via RDP.
Jun 29 – Jul 1	Lateral movement across 47 hosts via RDP and administrative SMB shares; redundant remote access tool deployed.
Jul 1	Domain Admin group membership obtained via nested group membership; persistence footholds established on DC-01, DC-02.
Jul 2 – Jul 4	Systematic collection of billing/HR/finance file shares; data staged into password-protected archives on FS-CORP-03.
Jul 4, 23:05 – Jul 5, 04:30	Staged archives (~340 GB) exfiltrated to external cloud storage over multiple throttled sessions.
Jul 5, late evening	Endpoint protection disabled on subset of hosts; Volume Shadow Copies deleted domain-wide via GPO and remote execution.
Jul 6, 03:12	Ransomware payload detonated across domain-joined endpoints/servers; .wardlck extension; README_RECOVER.txt dropped.
Jul 6, 05:50	Night-shift IT staff identify clinical documentation access failures; ransom note discovered.
Jul 6, 06:15	MHP activates internal Incident Response Plan; downtime procedures initiated.
Jul 6, 07:40	Aegis Cyber Response Group engaged; remote triage begins.
Jul 6, 14:00	Network segmentation actions completed to isolate affected VLANs.
Jul 7	Forensic imaging of priority hosts completed; malware samples submitted for analysis.
Jul 8	Scope substantially confirmed; credential reset of all domain/service/privileged accounts initiated.

Date/Time	Event
Jul 9 – Jul 11	Clean restoration of priority clinical systems from offline/immutable backups; DC rebuild.
Jul 13	Full restoration of clinical documentation access across all 12 sites confirmed.
Jul 16	Draft incident report issued to IR Steering Committee.

4. Technical Findings — Attack Narrative

4.1 Initial Compromise

The intrusion originated from a spearphishing email sent to a billing specialist within MHP's Revenue Cycle Management department. The email impersonated a known third-party clearinghouse vendor used by MHP for claims processing, using a look-alike sending domain that differed from the legitimate vendor domain by a single transposed character. The email contained an attached document styled as an overdue invoice reconciliation report.

Upon opening the attachment, the recipient was presented with a lure image instructing them to “Enable Content” to view the document correctly — a standard social engineering technique used to induce victims to enable macro execution, which is disabled by default in modern Office configurations. Macro execution triggered a short PowerShell command that downloaded and executed a second-stage payload from attacker infrastructure, establishing the initial foothold on host RCM-WKS-114.

Immediately following execution, the malware established two independent persistence mechanisms on the host: a scheduled task configured to re-launch the implant at regular intervals, and a registry Run key referencing a renamed copy of the payload stored in the user's local application data directory. This redundancy is a common technique among ransomware-affiliate operators to survive reboots and casual remediation attempts by defenders who have not identified the full persistence footprint.

4.2 Command and Control

Following the establishment of persistence, the compromised host began periodic outbound communication to attacker-controlled infrastructure over standard HTTPS on port 443, blending with legitimate encrypted web traffic. Traffic analysis conducted during the investigation identified a beaconing interval that varied between 45 and 90 seconds with randomized jitter, consistent with commercially available and open-source command-and-control frameworks commonly repurposed by ransomware affiliates. No DNS-based or custom protocol channels were identified during this phase; the actor relied exclusively on web-protocol-based communication for the duration of the intrusion, including during later-stage tool transfer and data exfiltration activity.

4.3 Internal Reconnaissance and Discovery

Over a three-day period following initial access, the threat actor conducted methodical reconnaissance of the MHP environment using native operating system utilities rather than custom tooling, a technique that substantially reduced the likelihood of signature-based detection. Observed activity included enumeration of local system configuration and installed software, enumeration of domain user accounts and their properties, identification of domain security groups and their membership (with particular attention to groups associated with IT administration and clinical systems support), and enumeration of reachable systems across the internal network to build a target list for subsequent lateral movement.

This reconnaissance activity allowed the actor to identify a legacy shared service account associated with a scanning/imaging integration tool, which had been provisioned several years prior with domain-level privileges broader than its function required — a common finding in healthcare environments where clinical integration accounts are often over-privileged to avoid troubleshooting integration failures.

4.4 Credential Access and Privilege Escalation

The threat actor obtained credential material through two distinct techniques. First, on the initial foothold host, the actor used a memory-dumping utility to extract credential material from the memory space of the Windows local security authority process, a widely used technique for harvesting cached credentials of any user who had recently logged onto the host, including a helpdesk technician who had remotely assisted the billing specialist several days prior.

Second, and more consequentially, the actor requested Kerberos service tickets associated with the previously identified over-privileged service account and captured the ticket material for offline password-cracking. Because the service account's password had not been rotated in over three years and did not meet current complexity requirements, the actor successfully recovered the plaintext password within approximately 48 hours using distributed cracking resources. This credential provided the actor with privileges sufficient to authenticate directly to additional internal systems and, through nested group memberships that had accumulated over time without periodic access review, ultimately provided a path to Domain Administrator-equivalent access.

4.5 Lateral Movement

Using the compromised privileged service account, the threat actor moved laterally across the environment primarily via Remote Desktop Protocol and administrative file-sharing protocols (hidden administrative shares), authenticating directly with valid, harvested credentials rather than relying on exploits — a pattern that is significantly harder to distinguish from legitimate administrative activity and which evaded MHP's signature-based endpoint detection tooling for the duration of the intrusion. Over a three-day window, the actor accessed 47 distinct hosts, including domain controllers, file servers, a backup management console, and a number of clinical workstation and imaging-adjacent systems located on a network segment that had not been fully isolated from the corporate IT VLAN due to a legacy routing exception supporting a third-party imaging vendor's remote support access.

To support this movement, the actor transferred a lightweight, legitimately-signed remote access utility to multiple hosts, providing redundant access channels independent of the original implant and increasing resilience against partial remediation.

4.6 Persistence Expansion and Defense Evasion

Upon reaching the domain controllers, the actor established additional scheduled-task-based persistence to ensure continued access even in the event that the original foothold host was remediated or the compromised service account credentials were rotated. In the hours preceding ransomware deployment, the actor took deliberate steps to reduce the organization's ability to detect and recover from the imminent impact phase: endpoint protection agents were disabled or uninstalled on a subset of hosts using administrative tooling, and Volume Shadow Copies — a Windows feature that can allow file-level recovery without full backup restoration — were systematically deleted across the domain using a combination of Group Policy-based script deployment and direct remote command execution against individual hosts.

4.7 Collection, Staging, and Exfiltration

In the days prior to encryption, the threat actor systematically accessed and copied data from file shares associated with the Revenue Cycle Management, Human Resources, and Finance departments. Rather than exfiltrating data directly from source locations, the actor staged collected files into a small number of password-protected compressed archives within a temporary directory on a corporate file server, a technique that both reduces the number of distinct exfiltration events required and complicates content-inspection-based

data loss prevention controls that were not configured to inspect encrypted archive contents.

The staged archives, totaling approximately 340 GB, were subsequently transferred to an external commercial cloud storage service over a period of roughly five and a half hours, split across multiple upload sessions and throttled to avoid triggering bandwidth-anomaly alerting thresholds that had been configured for the environment. Because the destination service uses standard HTTPS endpoints shared with a large volume of legitimate business traffic, the exfiltration activity was not flagged by MHP's web proxy or firewall logging at the time it occurred and was identified only during post-incident log review.

4.8 Impact — Ransomware Deployment

At approximately 03:12 on July 6, 2026, the threat actor executed the ransomware payload across domain-joined systems, leveraging the Domain Administrator-equivalent access obtained earlier in the intrusion to distribute and trigger the encryptor broadly rather than host-by-host. The encryption routine appended a distinctive file extension to affected files and dropped a ransom note in each affected directory directing MHP to a Tor-based negotiation portal and threatening publication of the previously exfiltrated data if payment was not made within a stated deadline. In parallel with encryption, a number of backup-related and database services were forcibly stopped on affected hosts prior to encryption, apparently in an effort to release file locks and maximize the proportion of data successfully encrypted, including on systems supporting clinical documentation workflows at four MHP sites.

5. Affected Systems and Assets

- 1,197 Windows endpoint workstations across corporate and clinical environments, encrypted.
- 18 Windows servers encrypted, including 2 of MHP's 4 domain controllers.
- 1 primary corporate file server (FS-CORP-03) used as the staging point for data exfiltration; contents fully encrypted post-exfiltration.
- 4 of 12 clinical sites experienced EHR/documentation system unavailability for 36–52 hours due to dependency on affected authentication and file infrastructure.
- Backup management console (BKUP-MGT-01) was accessed by the threat actor but backup repository integrity was preserved due to immutable/offline backup architecture for tier-1 clinical systems; corporate-tier backups suffered partial data loss for systems backed up only to online, domain-joined targets.
- Imaging-adjacent network segment (12 hosts) reachable due to legacy routing exception; no evidence of encryption or exfiltration activity on imaging modality devices themselves, though the segment was used as a pivot point during lateral movement.

6. Business and Operational Impact

Clinical operations at the four affected sites transitioned to downtime/paper-based documentation procedures for the duration of the outage, consistent with existing emergency preparedness protocols. No adverse patient safety events were reported as directly attributable to the outage; however, appointment rescheduling and delayed billing submission resulted in estimated short-term revenue cycle impact currently under assessment by the Finance department. Human Resources and Finance corporate functions experienced reduced productivity for approximately five business days while systems were restored in priority order behind clinical infrastructure. Legal and Compliance are separately assessing notification obligations related to the confirmed exfiltration of files containing patient billing information and employee personal information, in coordination with outside privacy counsel and MHP's cyber insurance carrier.

7. Root Cause Analysis

The investigation identified the following contributing factors, presented in approximate order of causal significance. The initial compromise succeeded because macro execution from an externally-sourced document was permitted for the affected user's role, and email filtering controls did not flag the spoofed look-alike sending domain. The intrusion escalated from a single-host compromise to a domain-wide event primarily because a shared service account was provisioned with privileges substantially exceeding operational need, had not been subject to periodic access review, and used a password that had not been rotated in over three years. Lateral movement was able to proceed largely undetected because the organization's detection tooling was heavily weighted toward signature and malware-based detection rather than behavioral detection of anomalous authentication and administrative-protocol usage, and because network segmentation between corporate IT and certain clinical-adjacent segments contained an undocumented legacy exception. Finally, the organization's ability to detect data exfiltration was limited by a lack of egress monitoring calibrated to detect archive-based, throttled transfers to common cloud storage destinations, and endpoint protection was capable of being disabled by an account holding local administrator rights without triggering a tamper-protection alert.

8. Containment, Eradication, and Recovery Actions

Upon engagement, Aegis Cyber Response Group and MHP IT Operations executed the following actions in sequence: isolation of confirmed and suspected affected network segments; forced credential reset for all domain user accounts, service accounts, and privileged accounts, with immediate deprecation of the over-privileged legacy service account identified in Section 4.4; deployment of enhanced endpoint detection tooling with tamper protection enabled across all reachable endpoints; forensic imaging and analysis of priority systems; rebuild of both affected domain controllers from known-good media; restoration of clinical systems from offline/immutable backup infrastructure, validated for integrity prior to reconnection; and phased reconnection of systems to the production network only after validation against indicators of compromise identified during the investigation. A full indicator list, including file hashes, network indicators, and host-based artifacts, is provided under separate, access-controlled cover to support detection engineering and threat intelligence sharing.

9. Indicators of Compromise (Summary)

The following is a representative, non-exhaustive summary of indicators identified during the investigation. This is a synthetic dataset generated for exercise purposes and does not correspond to any real infrastructure.

Type	Indicator	Context
File hash (SHA-256)	8f14e45fceeaa167a5a36dedd4bea2543dcbb2c3b9e1f5d0d3b4a2c1e9a7b6d40	Initial-stage macro loader
File hash (SHA-256)	3b5d5c3712955042212316173ccf37be5c47dcac82fe6d6dc4f96a3a58a9a67	Second-stage implant payload
File hash (SHA-256)	c0e63b3d2e5e1d3c9a2f8b4a6d7c1e0f9b8a3c2d1e0f7a6b5c4d3e2f1a0b9c88	Credential-dumping utility
Domain	claim-recon-verify[.]com	Spearphishing sender domain (look-alike)
Domain	update-cdn-relay[.]net	Command-and-control infrastructure
IP Address	185.220.101.47	C2 infrastructure
IP Address	194.35.112.9	Secondary C2 / data exfiltration relay
File path	%LOCALAPPDATA%\Microsoft\Sync\wsvc32.exe	Persistence payload location
Registry key	HKCU\Software\Microsoft\Windows\CurrentVersion\Run\WinSyncHelper	Persistence Run key
Scheduled task	\Microsoft\Windows\SystemSync\SyncMaintenance	Persistence task on initial host and domain controllers
Ransom note filename	README_RECOVER.txt	Dropped in encrypted directories
Encrypted file extension	.wardlck	Appended to encrypted files
Cloud storage endpoint	storage-relay-export[.]cloudapp-hosting[.]net	Exfiltration destination

10. Recommendations

MHP should prioritize a review and reduction of standing privileges held by service accounts, with a target of enforcing least-privilege access and instituting mandatory periodic credential rotation and access review for all non-interactive accounts. Email security controls should be enhanced to include look-alike domain detection and stricter default handling of macro-enabled attachments from external senders. Network segmentation between corporate/administrative systems and clinical-adjacent segments should be reviewed to identify and remediate legacy routing exceptions, with particular attention to third-party vendor remote access paths. Detection capability should be expanded to include behavioral analytics for anomalous authentication patterns, administrative protocol usage, and off-hours privileged account activity, rather than relying primarily on signature-based malware detection. Endpoint protection tooling should be reconfigured with tamper protection enabled and alerting on attempted service/agent termination. Egress monitoring should be enhanced to detect large or throttled data transfers to cloud storage destinations, including inspection or anomaly-detection coverage for encrypted archive traffic. Finally, MHP should validate that immutable or offline backup coverage is extended to all tier-1 and tier-2 systems, not solely clinical infrastructure, and should conduct a tabletop exercise within 90 days to validate updated incident response procedures against the findings of this incident.

11. Lessons Learned

The incident demonstrated that MHP's downtime procedures and clinical continuity planning performed effectively under real-world pressure, limiting patient safety impact despite significant system unavailability. It also demonstrated that reliance on signature-based detection left a substantial gap in visibility into credential-based and "living off the land" attacker activity that persisted undetected for eleven days. The presence of immutable backup infrastructure for clinical systems was a decisive factor in limiting recovery time and avoiding a ransom payment decision; extending this architecture organization-wide is the single highest-value structural improvement identified by this investigation. Finally, the incident reinforced the operational risk posed by legacy service accounts and undocumented network exceptions that accumulate over time in complex, multi-site healthcare environments, underscoring the need for recurring, rather than one-time, access and architecture review processes.

12. Appendices

Appendix A — Glossary

C2 (Command and Control): Infrastructure used by a threat actor to maintain remote communication with, and control over, compromised systems.

Dwell Time: The period between initial compromise and detection/eradication of a threat actor's presence in an environment.

EHR: Electronic Health Record system.

LOLBins: “Living off the Land Binaries” — legitimate, pre-installed system utilities repurposed by attackers to blend malicious activity with normal system operation.

RaaS: Ransomware-as-a-Service, an operating model in which ransomware developers license tooling to affiliates who conduct intrusions.

Appendix B — Report Distribution and Contacts

This report is distributed on a confidential, need-to-know basis to the Incident Response Steering Committee, MHP Executive Leadership, Legal Counsel, and MHP's cyber insurance carrier point of contact. Questions regarding this report should be directed to the MHP Chief Information Security Officer or the Aegis Cyber Response Group engagement lead.

Appendix C — Note on Report Nature

This document was generated as a synthetic exercise artifact to support validation of a MITRE ATT&CK technique-mapping methodology/tool. All entities, systems, indicators, and events are fictional. A corresponding technique reference list, enumerating the ATT&CK techniques intentionally reflected in the narrative above, has been provided as a separate document to support manual verification and is not part of the distributed report package in a real-world scenario.

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